Kelee Petzelt
Individual article

By Kelee Petzelt

You Can’t Improve Healthcare Access on a Broken Foundation: The Truth About Pharmacy Deserts

We talk a lot about improving healthcare access these days. It fills headlines, conference agendas, political talking points.

But if we’re being honest — you **can’t** improve healthcare access when the very system designed to provide it is broken, corrupted, and getting worse by the year.

Today, over **15.8 million Americans** live in what we call "pharmacy deserts" — areas where getting medication isn’t just inconvenient, it’s almost impossible. But instead of asking *how many pharmacy deserts* there are, it’s time to ask a better question:

> How many more have appeared since we first named the problem a decade ago?

The term "**pharmacy desert**" first entered the academic and public conversation in **2014**, when researchers at the University of Illinois called out growing gaps in pharmacy access — gaps tied to poverty, race, and healthcare inequality.

We named the problem. We studied the problem. We reported on the problem.

And yet, from 2014 to today, **more than 9,000 pharmacies have closed across the U.S.** Mostly in the places that needed them most: rural towns, urban neighborhoods, low-income communities.

Pharmacy deserts didn't shrink. They exploded.

If you listen to excuses, you'll hear talk about shrinking margins, tightening budgets, economic realities. But here's the truth:

While patients lost their local pharmacies, **PBMs and corporate healthcare giants raked in record profits.**

- **CVS Health** (which owns CVS Pharmacy and Caremark PBM): 2023 Net Income $8.3 billion, nearly doubling from $4.2 billion in 2022
- **UnitedHealth Group** (OptumRx PBM, UnitedHealthcare Insurance): 2023 Net Income $22.4 billion, up 11% from 2022
- **Cigna Group** (Express Scripts PBM): 2023 Net Income $5.2 billion, though down from $6.7 billion in 2022

There wasn't a shortage of money. There was a redistribution — upward, toward corporate conglomerates, and away from the small pharmacies, patients, and communities they claimed to serve.

While corporations profited, families were left without access to basic medications. While PBMs tightened their grip on pharmacy reimbursements, small towns watched their last remaining lifeline vanish.

I Lived It.

There were times I had to choose between my own medications and what my kids needed. I chose my kids — every time. Because when you live in a pharmacy desert, healthcare isn't a service. It's a gamble.

People like to talk about healthcare access in abstract numbers and percentages. But for millions of families, it's a brutal, everyday reality.

- Driving hours for an antibiotic.
- Waiting days for a life-saving medication that used to be down the street.
- Rationing pills because refills aren't reliably accessible.
- Choosing between medicine, groceries, and school supplies.

This is what pharmacy deserts look like up close. And no amount of buzzwords will fix that.

Measuring Deserts Doesn't Save Lives. Acting Does.

We've gotten good at measuring the social determinants of health. We know who is struggling — which zip codes have higher poverty, which neighborhoods have worse access, which towns are losing pharmacies the fastest.

But what good are statistics if we don't act on them?

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For too many in healthcare, social determinants of health have become **news stories** and **conference talking points** — drama to highlight, not devastation to fix.

For the families being measured, it's not academic. It's not a case study. It's survival.

They don't need another report telling them how sick or underserved they are. They need real action — funding, protection, and policy that restores access instead of just documenting its loss.

Compliance Isn't Rational in a Desert.

We love to talk about medication adherence. About patients "complying" with care plans.

But let's be honest: ***There's no rational adherence in a healthcare desert.***

- You can't pick up a prescription from a pharmacy that no longer exists.
- You can't manage a chronic illness if refilling life-saving medication requires taking a day off work and driving 50 miles.
- You can't stay on therapy when you're forced to choose between buying food, paying rent, or affording medicine.

Nonadherence isn't about irresponsibility. ***It's about inaccessibility.***

Until we fix the deserts, no amount of lectures about patient responsibility will matter. Let's Call It What It Is.

Pharmacy deserts didn't appear out of nowhere. They were created by:

- PBMs squeezing reimbursement to the bone.
- Chain pharmacy closures prioritizing profit over people.
- Government inaction that measured the problem but never addressed it.

And every year we keep "talking" about pharmacy deserts while doing nothing, **more Americans lose access to the healthcare they need to survive**.

So, Here's the Real Question:

> If we've known about pharmacy deserts since 2014, If we've watched them grow worse, If we've seen record profits for the corporations who helped create them...

Why has nothing changed?

Is it because solving the problem doesn't make anyone rich? Or because too many powerful players benefit from keeping it broken?

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Bottom Line:

You can’t fix healthcare deserts by studying them. You can’t fix pharmacy deserts by measuring them. You can’t fix access by preaching buzzwords while abandoning real communities.

If we truly want to improve healthcare access in America, We have to stop measuring the cracks — and start rebuilding the foundation.

Until then, patients will keep paying the price — and corporations will keep counting the profits.

> The next time someone publishes a report about “pharmacy deserts,” ask them one simple question:
> **How many more have appeared since we started talking about it — and when are we going to stop highlighting the pain — and actually fix it?**

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